

Public Health Emergency Operations Center "COUSP DRC"

MVE-17 Incident Management System

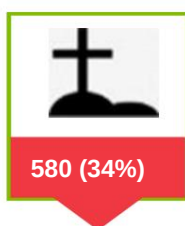
Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°053/MVB_06/07/2026

Affected provinces (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (37)	- Ituri (25/36): Aru, Aungba, Bambu, Boga, Bunia, Damas, Drodoro, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia, Fataki, Mandima, Lolwa and Boga. - North Kivu (11/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Masereka, Vuhovi, Mabalako and Musienene. - South Kivu (1/34): Miti-Murhesa
Reporting date	July 6, 2026
Publication date	July 7, 2026



Total confirmed cases
for the 3 provinces



Cumulative deaths
among confirmed cases
and case fatality rate



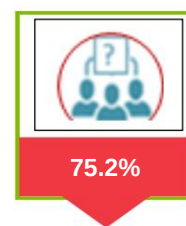
Patients in
isolation -
hospitalization



Cumulative number
of recoveries



Suspected cases of
the day



Contact tracing rates
for the 2 provinces

* Data updated following the DHIS2 database cleaning and harmonization process, including the consolidation of information from health zones, laboratories and case management structures.

Suspicious deaths occurring in the community are subject to investigation.

further investigation with a view to possible reclassification (Non-cases, probable cases)

1. HIGHLIGHTS

- A new health zone is assigned in the province of Ituri: Boga health zone.
- Continued strike by service providers in the health zones of Bunia and Rwampara.
- In the last 24 hours, **84 new confirmed cases and 59 deaths (84.7% community transmission)** were recorded in the provinces of Ituri (79 cases and 52 deaths) and North Kivu (5 cases and 7 deaths). The cumulative total now stands at 1,708 confirmed cases and 580 deaths, representing an overall case fatality rate of 34.0%.
- Nine (9)** deaths were recorded among patients with Ebola virus disease (EVD) in Ebola treatment centers in the provinces of Ituri (2 at CTE ISTM, 1 at CTE Mongbwalu, 1 at CTE Bunia and 1 at CT Sota) and North Kivu (2 at CTE Beni, 1 at CTE Butembo and 1 at CTE Katwa).
- Seven (7)** patients with Ebola virus disease in Ituri province (1 at CTE CME, 3 at CTE Mongbwalu, 1 at CTE ISTM and 2 at CT Logo) have been declared cured after obtaining negative control tests.

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

The Ituri province, located in the northeast of the Democratic Republic of Congo, shares a long and porous border with Uganda and South Sudan. For more than two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. Its population is estimated at over 8 million inhabitants, including more than one million internally displaced persons. The Mongbwalu health zone, considered the starting point of the epidemic, is located in the Djugu territory, 70 km from Bunia. This area is characterized by the presence of armed groups, frequent population movements towards Uganda, and numerous artisanal mining sites attracting migrant workers.

In September 2018, Ituri province was already affected by the 10th Ebola virus (EV) outbreak, which was then raging in North Kivu. The current outbreak, caused by Bundibugyo ebolavirus, was officially declared on May 15, 2026, by the Minister of Public Health. The three affected provinces have a total population estimated at nearly 15 million inhabitants, with massive internal displacement and cross-border flows to neighboring countries.

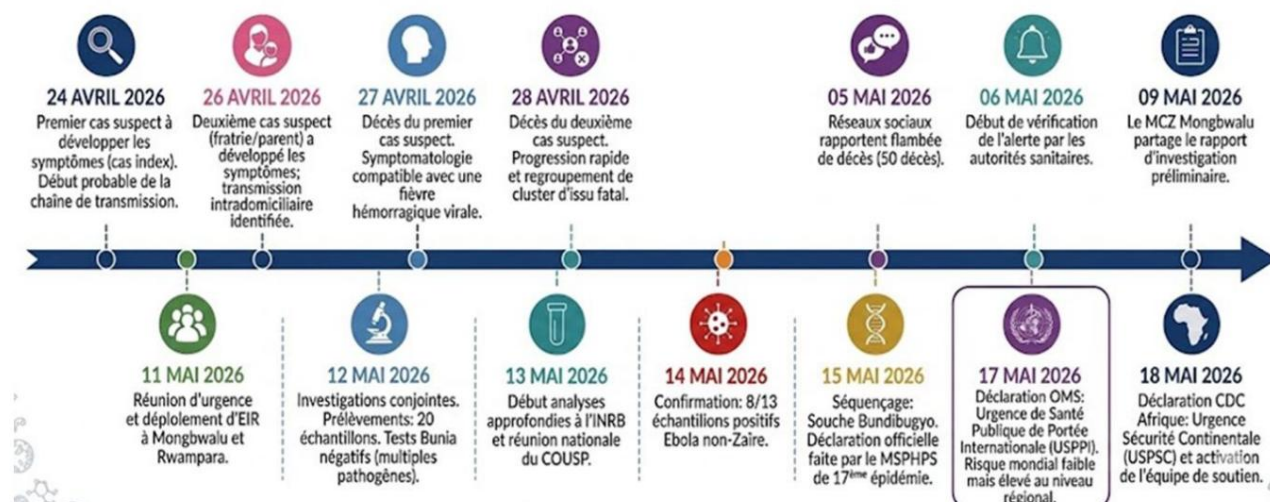


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

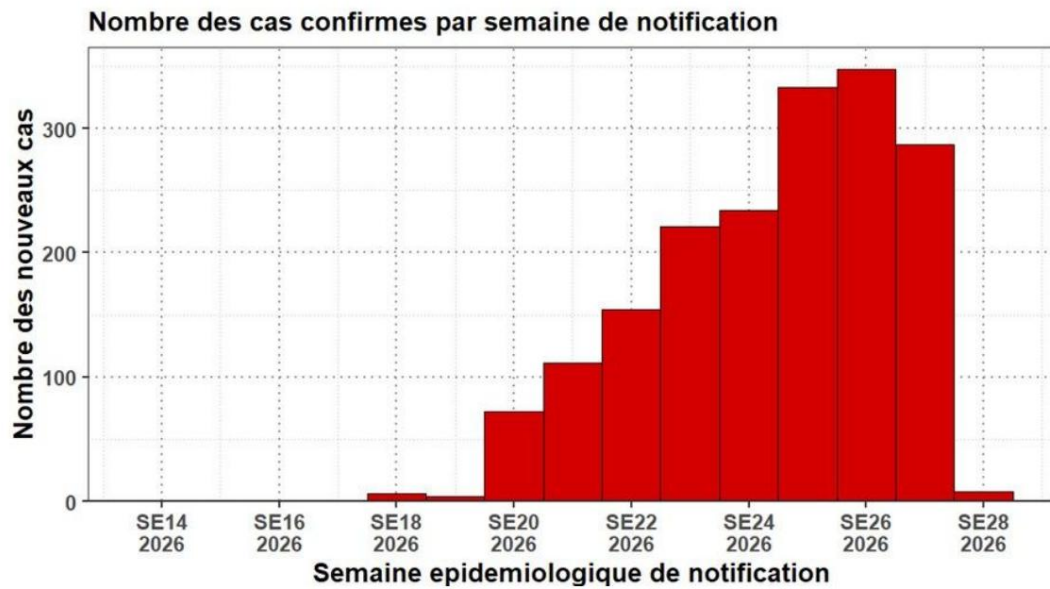
3.1 Spatial distribution and active hotspots as of July 5, 2026

Table 1. Distribution of confirmed cases and deaths by affected province as of July 6, 2026.

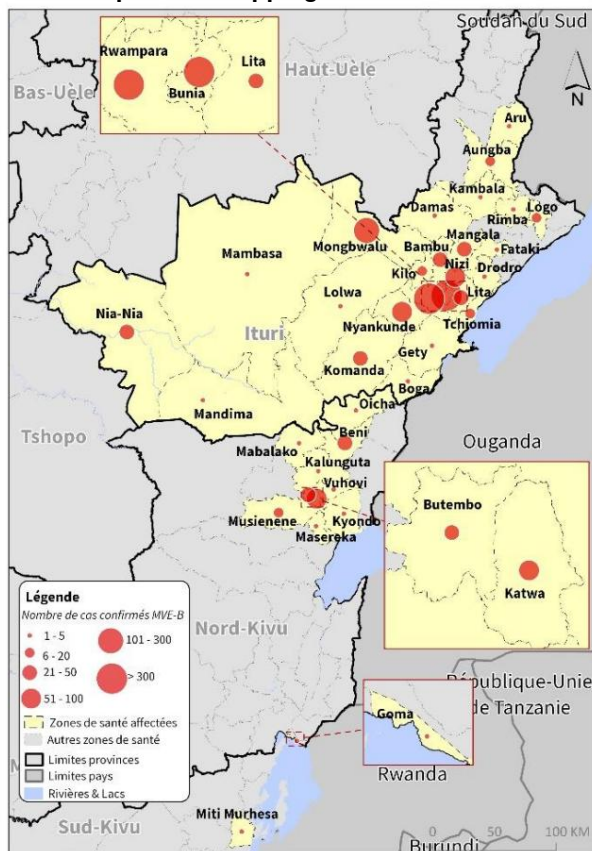
Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New confirmed cases (24h)
Ituri	1556	491*	31.5%	25 out of 36 (69.4%)	11
North Kivu	149	88*	59.1%	out of 34 (32.4%)	1 out
South Kivu	3	1	33.3%	of 34 (2.9%)	1,708
Total	37 out of 104 (35.6%)	580	34%	Deaths in Ituri and 4 in North Kivu were recorded among patients with Ebola virus disease (previous confirmed cases) in Ebola treatment centers.	

At the end of the day on July 6, 2026, a new health zone was affected in Ituri. The number of affected health zones rose to 37 out of 104 (section 3.1). Since May 26, 2026, the province of South Kivu has not recorded any confirmed cases, suggesting an absence of recent transmission.

3.2 Time-bound analysis: An increasing number of confirmed cases is observed from one week to the next, reflecting ongoing community transmission of the disease. The intensification of public health actions in epidemiological and biological surveillance (decentralization of diagnostic capabilities) has confirmed continued and increased community transmission in week 25. Compared to all weeks in which cases have been recorded since the beginning of the epidemic, weeks 25 and 26 continue to show a growing trend, with more than 300 cases per week.



3.3 Epidemic mapping



Figures 2 and 3. Spatial distribution of confirmed Ebola cases by affected health zones and in the three affected provinces as of July 6, 2026.

Table 2. Distribution of confirmed cases and deaths of Ebola virus disease by province and health zone (July 6, 2026)

Province / Health Zone	Cumulative confirmed cases (n)	Cumulative confirmed deaths (n)	Case fatality rate (CFR%)
ITURI	1556	491	31.6%
Bunia		134	28.4%
Rwampara	472,357	75	21.0%
Mongbwalu	298	154	51.7%
Nyankunde	96	20	20.8%
Nizi	84	24	28.6%
Lita	40	16	40.0%
Komanda	23	8	34.8%
Bamboo	28	9	32.1%
Kilo			11.1%
Mangala	9	1	51.3%
Tchomia	39 15	20 2	13.3%
Damascus	5	0	0.0%
Aungba	6	2	33.3%
Rimba	4	0	0.0%
Aru	3	1	33.3%
Mambasa	4		75.0%
Logo	7		0.0%
Drodoro	3		100.0%
Gety	1		0.0%
Kambala	2	30302	100.0%
Nia-Nia	33	12	36.4%
Fataki	3		33.3%
Mandima	5	1	40.0%
Lolwa	1	2 1	100.0%
Boga			100.0%
Other areas not yet identified	1 17	10	0.0
NORTH KIVU	144	81	56.3
Butembo	41	20	48.8%
Katwa	56	41	73.2%
Blessed	29	19	65.5%
Oicha	3	2	66.7%
Kalunguta	2		50.0%
Kyondo	4	1	50.0%
Goma		2	0.0%
Masereka	1	0	0.0%
Vuhovi	3 1	0 1	100.0%
Mabalako		0	0.0%
Musienene	1	2	25.0%
SOUTH KIVU	83	1	33.3
Miti-Murhesa	3	1	33.3
TOTAL	1,708	580	34.0%

Over the past 24 hours, **84 new confirmed cases and 59 deaths (84.7% community) have been recorded** in the provinces of Ituri (79 cases and 52 deaths) and North Kivu (5 cases and 7 deaths), highlighting the progression of the epidemiological situation of the 17th Ebola Virus Disease (EVD) epidemic in the Democratic Republic of Congo. Thus, the cumulative total stands at 1,708 confirmed cases and 580 deaths, expressing an overall case fatality rate of 34.0%.

Provincial dynamics and case concentration

The **Ituri** province alone accounts for **91.1%** of cases (n=1,556) and **84.7%** of deaths (n=491), representing a crude case fatality rate of **31.6%**, making it the epicenter of the epidemic. The health zones of Bunia (472 cases), Rwampara (357 cases), Mongbwalu (298 cases), Nyankunde (96 cases), and Nizi (84) remain the most affected, accounting for approximately 84.0% of reported cases at the provincial level and 76.5% at the national level. In Nizi, the epidemic is currently progressing more rapidly than in the other zones.

Severity of the epidemic in North Kivu: A total of 149 confirmed cases and 88 deaths have been reported in North Kivu province since the start of the epidemic, representing a **crude case fatality rate of 59.1%**. This high case fatality rate is concentrated in the health zones of **Katwa** (73.2%), **Oicha** (66.7%), **Beni** (65.5%), **Kalunguta**, and **Kyondo** (50.0%), respectively. The Katwa health zone has the highest number of reported cases (n=56), followed by the Butembo health zone (n=41) with a case fatality rate of 48.8%. A case fatality rate of 100% was observed in the exceptionally small Vuhovi health zone.

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

National / Ituri & Kinshasa

- Holding the coordination meeting with the response team and technical partners and financial support for field activities.

National level :

- Coordination meeting with an agenda: 100%
- Action items completed for daily coordination meetings: 80% **Provincial level :**

- Coordination meeting with an agenda: 100% for ITURI and North Kivu; South Kivu: ND
- Action items completed for daily coordination meetings: N/A for the three provinces (ITURI, North Kivu and South Kivu)

Zonal Level : •

- Coordination meeting with an agenda: N/A
- Action items completed for daily coordination meetings: N/A

4.2 Epidemiological surveillance

Table 3. Management of epidemiological alerts (24h) as of July 6, 2026

Indicator		Ituri	North Kivu	South Kivu	Total
Report (D-1)		62	0	4	66
New Alerts			726	10	1,150
Total alerts for today			726	14	1,216
Alerts investigated			726	14	963
Investigation rate			100.0%	100.0%	79.2%
Alerts confirmed	Alive	414,476,223	46.8%	1204	167
	Deceased	50	20	0	70
Total suspected cases for today		170	64	3	237

During the day of July 6, 2026, a total of **1216 alerts** were recorded, including **1150** new alerts, demonstrating the continued active epidemiological surveillance in the affected provinces.

The majority of alerts were reported in North Kivu (**726, 71.4%**), followed by Ituri (**414, 36.0%**) and South Kivu (**10, 0.9%**).

Of the alerts recorded today, **963 (79.2%)** were investigated. The investigation rate reached 100% in North Kivu and South Kivu, while it stood at **46.8%** in Ituri, highlighting a need to strengthen investigative capacities in this province.

The investigations led to the confirmation of 237 suspected cases, of which **167 (70.5%)** involved living individuals and **70 (29.5%)** community deaths. Ituri recorded the highest number of confirmed suspected cases with **170 (71.7%)**, followed by North Kivu with **64 (27.0%)** and South Kivu with **3 (1.2%)**.

Continued strengthening of surveillance, rapid investigation of alerts and early detection of cases, particularly in Ituri, remains essential to interrupt the chains of transmission.

Table 4. Follow-up of contacts of confirmed cases (1) as of July 6, 2026

Province	Contacts under follow-up (n)	Contacts viewed (n)	Follow-up rate (%)
Ituri*	9,404	6,864	72.9%
North Kivu	2,786	2,303	82.7%
Total	12,190	9,167	75.2%

* In Ituri, the monitoring rate was calculated only for health zones that had submitted reports (completeness: 75%). The health zones of Aru, Damas, Kambala, and Mandima did not have data available at the time of report consolidation.

As of July 6th, **12,190** contacts were under surveillance in the provinces of Ituri and North Kivu. Of these, 9,167 were actually seen, corresponding to an overall follow-up rate of **752%**.

Ituri had the highest number of contacts under follow-up, 9,404 (77.1% of the total), with 6,864 contacts seen, representing a follow-up rate of 73.0%. In North Kivu, 2,786 contacts were under follow-up, of which 2,303 (82.7%) were visited.

Contact tracing performance is comparable between the two provinces; however, the overall follow-up rate remains below the recommended operational threshold for optimal contact monitoring. Strengthening daily contact tracing, particularly of those who have not been seen, remains a priority to ensure the early detection of secondary cases and limit the risk of community transmission.

Table 5. Contact tracing of confirmed cases (2) as of July 6, 2026

Province	Number of new contacts		Number of contacts had become suspects		Number of contacts made in the last 21 days	
	n		n		n	%
Ituri	144	42.9 %	4	57.1 %		0.0%
North Kivu	192	57.1%	3	42.9%	0 422	100.0%
Total	336	100.0%	7	100.0%	422	100.0%

The day ended on July 6th with **336 new contacts** listed, the majority in Ituri (144, 42.9%), while North Kivu listed 192 (57.1%). Among the contacts under monitoring, eight (7) developed symptoms consistent with the definition of a suspected case, including four (0.03%) in Ituri and three (0.02%) in North Kivu, requiring immediate investigation and management in accordance with current procedures.

Furthermore, 422 contacts successfully completed their 21-day follow-up period without developing symptoms consistent with the disease in North Kivu.

These results demonstrate the continuation of contact tracing and follow-up activities, while highlighting the need to maintain rigorous surveillance to ensure early detection of secondary cases and to quickly interrupt any potential chain of transmission.

Update on checkpoint and point of entry (PoC/PoE) activities

- **Ituri** : Four (4) alerts were notified to the PoE/PoC, including three confirmed live alerts at the Mabakese, Zunguluka, and Poipo PoCs, and one body intercepted at the Mudzibala PoC. One live alert was referred to the CTE, while two were not referred due to resistance from the families. The body was recovered by the Bunia Health Zone team.
- **North Kivu**: Two (2) lifeless bodies were intercepted at the Kasitu and Mukulia PoCs and referred to the morgue from the Beni General Referral Hospital for sampling.

Table 6. Monitoring of indicators at Points of Entry and Points of Control (PoE/PoC), as of July 6, 2026

INDICATORS	Ituri	North Kivu
Proportion of PoC enabled and PoE enhanced	98.1% (54/55)	100.0% (14/14)
Number of people who went through PoE/PoC	99,548	59,346
testing, % of travelers screened,	94.3%	98.5%
% of travelers who washed their hands, % of	95.9%	98.5%
travelers made aware of the issue	99.7%	98.5%
Number of alerts notified	4	2
Number of validated live alerts	3	0
Number of lifeless bodies intercepted today	1	2
Number of problematic contacts intercepted today; %		0
of validated live alerts referred to CT/CTE; % of	0	0.0%
deceased bodies swabed	33.3%	100.0%
Number of positive results of referred suspected cases	100.0% 0	0

4.3. Laboratory

- **Ituri** : 172 samples collected and analyzed (**positivity 45.9%; n=79**).
- **North Kivu** : 77 samples collected and analyzed (**positivity 6.5%; n=5**).
- **South Kivu** : 3 samples collected and ongoing analysis.

4.4. Infection Prevention and Control (IPC)

Ituri: 11 households were identified, of which 9 were decontaminated around confirmed cases in the health zones of Rwampara, Komanda, Nyakunde, and Nizi, and two (2) were not decontaminated due to community resistance; no hygiene kits were distributed to these households. Five (5) health establishments (HEs) were identified around cases, of which four (4) were decontaminated. In addition, 21 HEs were monitored and supported, two (2) were provided with transportation, and two (2) ESS vehicles were decontaminated, 20 PPLs were trained, and 152 contractors were briefed during formative supervision sessions. Eight vehicles were received by the DPS for the EDS teams.

North Kivu: Briefing on handwashing and screening of 18 law enforcement officers within the various PLMs in the Beni health zone; closing of the briefing of three EDS actor teams, composed of 12 people each, with the support of WHO in the Beni health zone; evaluation of the scorecard in 73 ESS, including 22 in Beni, 2 in Butembo, 10 in Kalunguta, 3 in Katwa, 14 in Lubero, 13 in Mabalako, 3 in Mutwanga and 7 in Oïcha.

Of these health assessments, 30 obtained a score below 50%, for an overall average score of 24.6%. One (1) death alert was recorded in Butembo and six (6) others in Oïcha, five of which came from the HGRO morgue in the Tenambo district and one from the Eringeti health center. Six (6) bodies were recovered, of which four (4) were secured.

4.5 Holistic care

Ituri:

- **Nine (9)** deaths were recorded among patients with Ebola virus disease in Ebola treatment centers in the provinces of Ituri (2 at Ebola Treatment Center ISTM, 1 at Ebola Treatment Center Mongbwalu, 1 at Ebola Treatment Center Bunia and 1 at Ebola Treatment Center Sota) and North Kivu (2 at Ebola Treatment Center Beni, 1 at Ebola Treatment Center Butembo and 1 at Ebola Treatment Center Katwa).
- **Seven (7)** patients with Ebola virus disease in Ituri province (1 at CTE CME, 3 at CTE Mongbwalu, 1 at CTE ISTM and 2 at CT Logo) have been declared cured after obtaining negative control tests.
- By the end of the day on July 6, 883 hot meals had been served, including 151 to suspected cases, 113 of the confirmed cases, 581 to PPL and 38 to accompanying persons.

Patient movement:

- No transfers to a general referral hospital were reported. Two (2) new admissions involved PPL and are included in the 67 admissions recorded during the day.

North Kivu:

- One (1) confirmed case escaped in the Musienene health zone before being transferred to the Katwa CTE.

Table 7. Patient movement within healthcare facilities (CTE/CT/CI) as of July 6, 2026

Indicator	Ituri	North Kivu	South Kivu	Together
Patients in bed (Day -1)	485	182	21	688
Total admissions (24h)	69	30	3	102
Exits (24h)				
Deceased (Suspects/Confessions)	13	4	0	17
No case	26	50	0	76
Healed	7	0	0	7
Escapees (Suspects/Confessions)	1	2	0	3
Transferred to the HGR	0	0	0	0
Total outflows	46	56	0	102
Patients in isolation (End of Day),	508	148	24	680
including confirmed cases	200	18	0	218
(NC+AC) and suspected cases	308	138	24	470
Overall occupancy rate	84.0%	115.0%	53.3%	84.1%

In total, 680 patients were in isolation, including 218 confirmed cases and 470 suspected cases, for an overall bed occupancy rate of **84.1%**.

Mental health activities and psychosocial support

Table 8. Key indicators of mental health and psychosocial support activities as of July 6, 2026

KEY INDICATORS	ITURI	NORTH KIVU	SOUTH KIVU
New confirmed cases admitted to the CTE who benefited from SMSPS interventions	5 (41.7%)	0	0
Confirmed cases currently being monitored that have received interventions SMSPS	61 (41.8%)	7	0
New suspected cases that benefited from SMSPS interventions	8 (22.9%)	3	1
Suspected cases under follow-up that have received interventions SMSPS	70 (52.2%)	5	3 (100.0%)
Laboratory results have been announced.	7	4	0
Including positive ones	6	0	0
Number of children separated in daycare who benefited from interventions SMSPS	1	0	0
Number of children separated in the community who benefited from SMSPS interventions	0	0	5
Number of accompanying persons at the CTE who benefited from interventions SMSPS	38	2	0
Number of PPLs who benefited from SMSPS interventions	28	2	0
Number of household and community members who benefited from SMSPS interventions	31	0	0
Number of EDS supported	6	0	0

4.6 Continuity of care

- Provision of essential medicines for PMA and PCA in other ZS.
- Support for non-cases (children, mothers and other vulnerable people) within the framework of continuity of services actions; Psychosocial support for affected PPLs and their dependents; Effective implementation of free healthcare.

4.7 Risk communication and community engagement

In Ituri, 9,144 people were reached through awareness messages during home visits to 6,500 households; 3,388 people were reached through educational talks and 1,689 through mass awareness campaigns in the health zones of Bunia, Nyakunde, and Aungba. Three advocacy meetings mobilized 25 people, while 85 slaughterers from the Bunia abattoir were trained on zoonoses and Ebola virus disease prevention measures. Six radio programs and six public service announcements were produced and broadcast; 36 community radio stations were engaged, and 15 information, education, and communication (IEC) materials were distributed.

Community feedback: 18 feedback items and infodemics were collected, of which 6 were clarified, representing 33.3%. The main concerns related to the late intervention of the EDS teams, fatigue linked to repeated awareness campaigns, and the perception of the response being used for political purposes.

Support for the pillars of the response

Surveillance: Support for resistance management, referral of patients to FOSA/CTE and awareness-raising of 32 family members of four suspected cases around CTE/CME on isolation and sample collection.

CREC: Continued awareness-raising activities in households, markets, public places and around CTEs, particularly in the health zones of Bunia, Nyakunde, Aungba and Rwampara.

North Kivu: Capacity building of 50 leaders and 150 community relays, members of the CACs of 10 high-risk AS in the Katwa health zone, on CREC, community-based surveillance and promotion of prevention measures against Ebola; Capacity building of 36 members of the EDS pillar on interpersonal communication during interventions, in order to persuade families and communities to adhere to community protection interventions.

4.8. Logistics

Ituri

- Mobility guaranteed for the different pillars of the response.

North Kivu:

- Receiving computer equipment (10 computers, 4 overhead projectors and 61 phones with chargers), donated by partner RELIEF LIFE; unpacking medicines, donated by partner UG-PDSS; receiving and unpacking PCI inputs, donated by partner WHO; receiving inputs, donated by partner UNICEF; preparing motorcycle delivery slips to the health zones of Beni, Oicha, Mabalako and Kalunguta.

4.9. Security

- Arson at the Kitatumba CTE (damage reduced thanks to police intervention) in North Kivu.
- Continued strike by service providers in the health zones of Bunia and Rwampara in Ituri.

4.10. PSEA (Prevention of Sexual Exploitation and Abuse)

Ituri : Briefing of stakeholders and community awareness activities on the prevention of sexual exploitation and abuse continued in the health zones of Bunia, Rwampara, Mongbwalu and Aru.

5. MAIN CHALLENGES, IMPACTS AND REQUIRED ACTIONS

Challenge	Impact	Actions required
No. 1 Reluctance to undergo post-mortem sampling (EDS/swabs) and community resistance	Confirmation delays, underestimation of cases, disruption of key activities	Intensify CREC, involve religious and community leaders, strengthen team security, and increase the number of EDS teams.
2 Insufficient capacity of CTEs and saturation in North Kivu (115.0%)	Admission delays, increased risk of nosocomial transmission	Accelerate the construction and expansion of CTEs, deploy additional temporary structures, and improve laboratory turnaround times.
3 Contact tracking performance not optimal (75.2% vs target 95%)	Uninterrupted transmission chains	Train, deploy and motivate community liaisons (RECOs), enhanced supervision
4 Low reporting of alerts in some affected areas	Undetected transmission, silent spread of the epidemic	Strengthen community surveillance, improve the reporting and rapid investigation of alerts
5 confirmed cases not yet broken down by health zone (17 cases)	Major gaps in the reconstruction of transmission chains	Conduct a data reconciliation, prune any possible duplicates, and reclassify or clarify the health zone and status (deceased, recovered, active, etc.) Active field verification
6 Insufficient supply of MEG and medical inputs	Weakening of overall care	Advocacy and urgent supply of essential medicines
7 Insufficient PCI inputs (PPE, chlorine) and limited training	High risk of nosocomial infections (96 HCW cases including 19 deaths, case fatality rate 19.7%)	Urgent supply, strengthening of IPC training
8 Stock of samples not yet analyzed (backlog) at the laboratory North Kivu and late diagnosis	Delayed confirmation, high lethality (56.3%)	Deploy a laboratory catch-up plan, strengthen diagnostic capabilities, improve the flow of laboratory data
9 Insufficient number of ambulances and isolation facilities (gap of 20 centers)	Transfer delays and isolation increase the risk of transmission	Urgent deployment of additional logistical resources and infrastructure
10 Insufficient and weak coordination use of COUSP frames	Operational inefficiency and delayed decisions	Strengthen coordination mechanisms at all levels, and make provincial COUSPs operational.
11. Insufficient resources financial (gap of 20 million USD)	Limitation of the implementation of all pillars of the response	Urgent mobilization of resources, advocacy with partners
12 Insecurity and limited access (CODECO, ADF)	Restrictions on operations, limited access to high-risk areas	Strengthening security coordination and humanitarian access
13 Continuity of essential services (CEHS) compromised	Increase in indirect mortality (not Ebola)	Effective implementation of free healthcare, strengthening of CEHS
14. High population mobility	High risk of rapid geographic expansion	Strengthening surveillance at PoE/PoC sites and cross-border coordination

6. PHOTOS OF FIELD ACTIVITIES



Arson at the Kitatumba CTE in North Kivu

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Avec l'appui technique de l'Organisation Mondiale
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